DEBTAN Tablet

Debtan is an oral anti-diabetic preparation with a potent hypoglycaemic effect. In certain form of diabetes, it can control disturbed carbohydrate metabolism. Glibenclamide, the active substance of Debtan, is a Sulfonylurea derivative, and is as well tolerated as Tolbutamide and as powerful as Chlorpropamide. Debtan mobilizes endogenous insulin and it is particularly suitable for oral therapy of maturity-onset diabetes. It brings a metabolic control for a period of about 24 hours.

Ingredient(s):
Each tablet contains:
Glibenclamide ……………………………………………………. 5mg

Indication(s):
Treatment of non-insulin dependent diabetes mellitus who respond inadequately to dietary measures alone.

Dosage and Administration:
Glibenclamide should be taken with or immediately after food. The total daily dosage is preferably given as a single dose at breakfast or with the first main meal, but due consideration should be given to the patient's meal habits and daily activity when apportioning dosage:-

New diabetics
In maturity-onset diabetes of mild to moderate severity treatment should be started with 5mg daily or 2.5mg in debilitated or elderly patients. If this dosage is not sufficient for proper control, it should be increased by 2.5mg at interval of one week or as directed by clinician. The total daily dose of glibenclamide rarely exceed 15mg. Increasing dosage beyond this level is unlikely to produce further response.

Initial dosage in patients transferred from other antidiabetic agents:-

(i) Sulphonylureas:
The switch to glibenclamide can be immediate. An initial dose of 10mg should not be exceeded. Subsequent dosage adjustments are based on the patient's blood or urine glucose response. If patients were on chlorpropamide they should be closely monitored for hypoglycaemia for the first 2 weeks of the transition.

(ii) Biguanides:
2.5mg glibenclamide can be substituted for the biguanide initially and adjusted as required after 3 to 5 days. On occasion, glibenclamide may be prescribed together with metformin if control is not adequate with either agent alone. Clinical benefit of the combination should be monitored from time to time.

Contraindication(s):
Severe metabolic decomposition with acidosis, precomatose states and diabetic coma, severe renal dysfunction, diabetes mellitus in young people and pregnancy.

Precaution (s) / Warning(s):
1. To be administered according to doctor's prescription.
2. Any adverse effects (hypoglycaemia manifestations, fever, skin rashes, nausea or other symptoms) occurring during treatment, should be reported to the attending physician immediately.
3. First sign of pregnancy must also be reported to the doctor without delaying, because a change to insulin and / or dietary treatment is necessary.
4. In rare cases, intolerance to alcohol may occur.

Side Effect(s) / Adverse Reaction(s):
Among the mild adverse effects observed with Sulphonylureas are gastrointestinal disturbance such as nausea, vomiting, heartburn, anorexia, constipation, diarrhea, and metal taste, and there may be headache, dizziness, weakness, paresthesia and tinnitus. These effects are usually dose-dependent. Skin rashes and pruritus may occur and photosensitivity has been reported. Rashes are usually allergic and may progress to more serious disorders. Changes of hemophilic system (e.g. decrease in the number of platelets, white and red blood cells) are rare.

Interaction with Other Medicaments

1. There may be an undesirable potentiation or attenuation of the blood-sugar-lowering action of glibenclamide when taken at the same time with alcohol or other drugs. Therefore, other drugs should be taken only with the doctor's approval or on his prescription.
2. Drugs that may potentiate the hypoglycemic action of glibenclamide e.g. β-receptor blockers, bezafibrate, biguanides, chloramphenicol, clofibrate, coumarin derivatives, fenfluramine, MAOIs, pentoxifyline in high parenteral dosage, phenylbutazone, phenyramidol, phosphamides, salicylates, sulfinpyrazone, sulfonamides, tetracyclines.
3. Attenuation of the hypoglycaemic action of glibenclamide may be caused by: Abuse of laxatives, corticosteroids, nicotinic acid in high doses, oestrogens, gestagens, phenothiazines derivatives, saluretics, sympathomimetic agents, thyroid hormones.
4. The hypoglycaemic action of oral-diabetic agents including glibenclamide may be enhance by sulphonamides, salicylates, phenylbutazone, coumarine derivatives, beta-blocking agents, monoamine oxidase inhibitors, cyclophosphamide, bezafibrate, clofibrate, fenfluramine, tetracyclines, sulphipyrazone and chloramphenicol. Conversely, thiazide diuretics, furosemide, ethacrynic acid, oral contraceptives containing oestrogens/gestagens, phenothiazine derivatives, nicotinic acid (high-dosage) sympathomimetics, thyroid hormones and corticosteroids may diminish hypoglycaemic activity. Hypoglycaemic activity may also be affected by tuberculostatics.

Pregnancy and Lactation

Glibenclamide must not be taken during pregnancy. The patient must change over to insulin during pregnancy. Patients planning a pregnancy must inform their doctor. Such patients should change over to insulin.
To prevent possible ingestion with the breast milk, glibenclamide must not be taken by breast-feeding women. If necessary, the patient must change over to insulin, or must stop breast-feeding.

Symptoms and Treatment for Overdosage, and Antidote(s):

Overdosage may result in hypoglycaemia. If symptoms of mild hypoglycaemia appear, patient should eat or drink a source of sugar and obtain medical assistance right away. Adjustment in oral antidiabetic dosage and / or meal patterns may be required. Good sources of sugar are orange juice, corn syrup, honey syrup cubes, or table sugar (dissolved in water). A popular source of sugar is a glassful of orange juice containing 2 or 3 teaspoonfuls of table sugar. Severe hypoglycaemic reactions with coma, seizures, or other neurological impairment can occur. If hypoglycaemic coma is diagnosed or suspected, the patient should be given a rapid intravenous injection of 50% glucose solution. This should be followed by a continuous infusion of a 10% glucose solution at a rate which will maintain the blood glucose at a level above 100mg/dl. Patients should be closely monitored for at least 24 to 48 hours in the hospital.

Shelf-Life:

3 years from the date of manufacture.

Storage Condition(s):

Keep in a tight container. Store at temperature below 30°C.
Protect from light and moisture.

Product Description and Packing(s):

A white color rounded rectangle tablet with scores at both sides.
Plastic bottle of 1000's
Blister packing of 10's x 10 and 10's x 100.

Y. S. P.®

Manufacturer: **Y.S.P. INDUSTRIES (M) SDN. BHD.**

Factory : Lot 3, 5 & 7, Jalan P/7, Section 13,
Kawasan Perindustrian Bandar Baru Bangi,
43000 Kajang, Selangor Darul Ehsan, Malaysia.
Tel: 03-89251215 (4 Lines) Fax: 03-89251298.

Office : No. 18, Jalan Wan Kadir, Taman Tun Dr. Ismail,
60000 Kuala Lumpur, Malaysia.
Tel: 03-77276390 (12 Lines)
Ordering line: 1 800 88 3027
Product line: 1 800 88 3679

Date of Revision: 16 February 2009